A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 April 2026

Dr Timothy Chew
Turn The Corner Medical Clinic

Dear Dr Chew,

Re: Miss Sisana Lazarus
DOB: 2001-03-13
Address: 8/113 Victoria Street, BRUNSWICK, VIC, 3056

Summary

- **Chronic nausea, altered bowel habit, and intermittent dysphagia.**
- **For gastroscopy and colonoscopy to exclude eosinophilic oesophagitis and investigate a family history of gastrointestinal malignancy.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Sisana, thank you for referring her.

Symptoms and history of presenting complaint

Sisana presents with a two-year history of gastrointestinal distress, which followed an acute episode of cannabinoid hyperemesis syndrome (CHS) in 2022. While her cannabis use has significantly reduced to infrequent use, her symptoms have persisted. She reports weekly episodes of nausea and vomiting, often triggered by intense exercise. This is accompanied by significant abdominal pain, described as sharp lower-quadrant cramping that typically precedes bowel movements. Her bowel habit is predominantly loose, with two to five movements per day, though she has recently noted very dark stools and occasional rectal bleeding during her menses, which she believes may be from the bowel.

Furthermore, Sisana reports intermittent mid-chest dysphagia, particularly when consuming dry textures such as bread. She notes a sensation of food boluses "bumping" in her chest, occasionally leading to regurgitation. She has been working with a pelvic floor physiotherapist for nine months to manage bladder overactivity and suspected pelvic floor dyssynergia; however, despite some improvement in stool frequency, her core symptoms remain unresolved.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Diet

Sisana consumes a diet including oats, avocado, chicken, and rice, but acknowledges she is a fast eater. Her intake includes daily coffee with full cream milk and approximately two glasses of artificially sweetened soft drinks (Coke Zero/Pepsi Max). I have advised her that the artificial sweeteners in these beverages may be contributing to her diarrhoeal symptoms via an osmotic effect.

Past medical history and medications

Sisana's medical history is significant for adenomyosis, characterized by heavy and painful periods, and chronic migraines. She has a history of atopy, specifically eczema, which she currently manages with topical tacrolimus and other creams. Her mental health history includes depression and anxiety, for which she is currently prescribed sertraline 100mg daily. She has previously used various aperients, including (Osmolax), senna, and bisacodyl/docusate (Coloxyl), with mixed results.

Social history, family history

Sisana is a graduate student currently completing her Master's thesis in creative writing. She lives alone and does not smoke. There is a significant family history of gastrointestinal issues; her father was treated for an intestinal malignancy (either gastric or colorectal) in his youth and undergoes regular surveillance. Both her father and brother suffer from severe gastro-oesophageal reflux disease.

Examination

On examination, Sisana appeared well. There was evidence of active eczema on her hands. Abdominal palpation revealed mild tenderness in the lower quadrants, but there were no palpable masses, organomegaly, or signs of peritonitis. An oral examination was unremarkable.

Impression and plan

The clinical picture is complex, likely involving a combination of functional bowel symptoms and structural or inflammatory considerations. Her history of atopy and intermittent dysphagia is suspicious for eosinophilic oesophagitis. Additionally, her altered bowel habit, perimenstrual bleeding, and family history of early-onset gastrointestinal malignancy necessitate an endoscopic evaluation.

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We discussed the following plan:

- **Investigations:** I have recommended a combined gastroscopy and colonoscopy with multiple biopsies. This will serve to exclude eosinophilic oesophagitis and investigate the lower gastrointestinal symptoms and family history.
- **Procedural Options:** We discussed the options of proceeding via the private system, a lower-cost private day centre, or the public hospital waitlist (I think the waiting list is acceptable for her symptoms). Sisana will confirm her preference after discussing these options with her family, as she does not have private insurance. .
- **Conservative Management:** In the interim, I have suggested she trial a fibre supplement such as psyllium husk (Metamucil) or partially hydrolysed guar gum (PHGG) to regulate stool consistency. I have also advised her to cease consumption of artificially sweetened beverages.
- **Follow-up:** We will review her progress and biopsy results approximately two weeks following her procedures. If symptoms persist despite normal findings, we may consider a low-dose neuromodulator such as nortriptyline - which I believe you have already prescribed which I think is correct here, though mindful of her concurrent sertraline use.

Thank you again for referring Sisana

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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